

Clearwater Internal Medicine — Diabetes Management Program

QUARTERLY DIABETES MONITORING LABS

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Franklin D. Nguyen	DOB: 06/29/1963	MRN: 1182734
Date: October 8, 2024 — Fasting	Provider: Dr. Sandra Levy, MD, CDE — Endocrinology	

GLYCEMIC CONTROL

Test	Result	Units	Reference	Flag
HbA1c (Current)	8.2	%	<7.0 (diabetic goal)	H
HbA1c (6 months ago)	7.9	%	<7.0	H
HbA1c (12 months ago)	7.6	%	<7.0	H
Fasting Glucose	198	mg/dL	70-130 (diabetic)	H
C-Peptide	1.8	ng/mL	0.8-3.1	
Fasting Insulin	18	uIU/mL	2.6-24.9	

RENAL FUNCTION AND URINALYSIS

Test	Result	Units	Reference	Flag
Creatinine (Serum)	1.05	mg/dL	0.74-1.35	
eGFR	74	mL/min/1.73m ²	>60	
Urine Albumin	42	mg/g creatinine	<30	H
Urine Creatinine	88	mg/dL	—	
Urine Glucose	Positive	—	Negative	H
Urine pH	6.0	—	4.5-8.0	
Urine Protein	Trace	—	Negative	A

LIPIDS AND CARDIOVASCULAR RISK

Test	Result	Units	Reference	Flag
LDL Cholesterol	72	mg/dL	<70 (diabetic)	H
HDL Cholesterol	44	mg/dL	>40 (M)	
Triglycerides	218	mg/dL	<150	H
Blood Pressure	144/88	mmHg	<130/80 (DM)	H

ASSESSMENT AND PLAN

HbA1c is 8.2%, worsening from 7.9% (6 months ago) and 7.6% (12 months ago). This upward trend despite current regimen (Metformin 1000mg BID + Glipizide 10mg BID) indicates inadequate glycemic control. Recommend adding a GLP-1 receptor agonist (semaglutide 0.5mg weekly) to the regimen — this will also address elevated triglycerides and provide cardiovascular benefit.

Microalbuminuria (urine albumin 42 mg/g) indicates early diabetic nephropathy. Initiating ACE inhibitor (Lisinopril 5mg daily) for renoprotection. Recheck urine albumin in 3 months.

LDL slightly above goal despite atorvastatin 40mg. Increasing to 80mg for more aggressive lipid management given diabetic cardiovascular risk. Triglycerides elevated — will reassess after glycemic improvement and GLP-1 initiation.

Blood pressure above goal. Adding Amlodipine 5mg daily and increasing home monitoring frequency to twice weekly. Return in 3 months for full reassessment.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.